Fax cover sheet

Westbay Community Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Priya Raman, LCSW
Phone: 555-0190
Date: 10/06/2026 10:15
Pages (including cover): 11
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

35M L leg laceration, sutured, medically cleared.
Requesting recuperative care for wound healing. H&P, nursing, labs and med list attached.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH • HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Westbay Community Hospital (fictional) Dept/Floor: Observation Unit
SW/CM/RN: Priya Raman, LCSW Phone/Pager #: 555-0190
Email: praman@westbay.example Nursing Station #: 555-0191
Authorized by: Dr. E. Mwangi Phone #:

Patient Information

Patient Name: Terrence Doe MRN: WB-2093368 Date of Birth (dd/mm/yyy): 03/21/1991
Gender: ☒ Male ☐ Female ☐ Transgender ☐ Other Ethnicity: White
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Laceration L lower leg, sutured
Admit Date: 10/04/2026 Any Wounds? ☒ Yes ☐ No
If yes, explain the number, location, size and stage:
L anterior shin laceration 7 cm, 9 sutures
Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 10/04/2026 Results: CXR negative

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* X No	Active Infection	Yes* X No
*Explain		*Explain	
Self Administer All Meds	X Yes No*	Ambulatory	X Yes No
Continent of Bowel & Bladder	X Yes No	Assistive Device	Yes* X No
Colostomy/Ileostomy	Yes X No	*Explain	
Foley Catheter	Yes X No	Hospice	Yes* X No
Diabetes	Yes X No	*Explain	
Insulin	Yes X No	Car	Yes X No
Diabetes related Oral Meds	Yes X No	Service Animal	Yes X No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? X No Yes
☒ Mental Health DX If checked, what type?
☒ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain

Length of Stay Authorized in Recupertive Care: 10 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Terrence Doe

DOB: 03/21/1991

Member ID: OCH-39917260

Referring Provider

Facility: Westbay Community Hospital (fictional)

Clinical

Primary Diagnosis: Laceration without foreign body, left lower leg

ICD-10: S81.812A

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 10/06/2026

Requested Days: 10

Plan Use Only

Status: Pending review

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Patient face sheet

Westbay Community Hospital (fictional) - PATIENT FACE SHEET

Patient: Terrence Doe
MRN: WB-2093368
Account #: 99093368
DOB: 03/21/1991
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 10/04/2026 23:50 (ED arrival 10/04/2026 21:15)
Admit Type: Inpatient
Unit/Bed: OBS-07
Attending: Esther Mwangi, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical

HISTORY AND PHYSICAL

Date of Service: 10/05/2026 00:30

Attending: Esther Mwangi, MD (Hospital Medicine)

HPI

35 y/o M with schizophrenia, unhoused x 3 years, brought by police after found with a laceration to the L shin from climbing a fence. Laceration irrigated and closed in ED with 9 sutures. Pt reports he stopped his medications "a few months ago". Admitted to observation for wound care and psychiatric evaluation.

PMH

- Schizophrenia (dx age 22), prior psychiatric hospitalizations x 3

Social

Unhoused x 3 years. Smokes 1 ppd. Denies alcohol. Cannabis daily.

Allergies

Haloperidol (dystonia)

Exam

VS: T 98.6F, HR 92, BP 128/80, RR 16, SpO2 99% RA

L anterior shin: 7 cm linear laceration closed with 9 sutures, edges approximated, no erythema.

Neuro: nonfocal. Alert, oriented to person and place only.

Psych: guarded, responding to internal stimuli

Plan

1. Laceration - daily dressing, suture removal in 10-14 days; Tdap given
2. Psychiatry consult

Isolation: None - standard precautions

Electronically signed: Esther Mwangi, MD 10/05/2026 01:10

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Psychiatry consultation

PSYCHIATRY CONSULTATION

Date: 10/05/2026 14:00

Consultant: H. Lindgren, MD (Psychiatry)

Reason: schizophrenia off medications, responding to internal stimuli

Interview

Pt endorses hearing voices for several weeks. Reports command auditory hallucinations telling him to "go out and hide from the cameras" and to "not trust anyone who gives you pills".

Denies suicidal ideation. Denies homicidal ideation. Denies visual hallucinations.

Off risperidone for about 4 months. Declines to restart antipsychotic today.

MSE

Disheveled, guarded, poor eye contact. Thought process: tangential. Thought content: paranoid delusions about surveillance. Perception: active command auditory hallucinations. Insight poor, judgment impaired.

Oriented to person and place, not to date.

Assessment

Schizophrenia, acute exacerbation, nonadherent to medications. Does not currently meet 5150 criteria.

Recommendations

- Encourage restart of risperidone; patient declining
- Referral to county outpatient mental health
- Not psychiatrically stable at this time

Signed: H. Lindgren, MD

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Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#2

Date: 10/06/2026 07:30

S: Wants to leave. Refused morning medications offered by psychiatry.

O: T 98.4F HR 84 BP 124/76 RR 16 SpO2 99% RA

L shin laceration clean, dry, intact sutures, no signs of infection.

A/P:

- L shin laceration - medically cleared for discharge. Suture removal 10-14 days.
- Schizophrenia off meds - per psychiatry, not stable; declines risperidone

Signed: Esther Mwangi, MD

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Nursing note

NURSING NOTE

Date: 10/05/2026 20:00
Pt observed talking to himself, appears to be responding to voices.
Oriented to person and place only.
Pt refused evening risperidone offered per psychiatry.
Ambulating in room and hallway independently, steady.
Independent with toileting and showering.
Not aggressive toward staff; guarded, keeps to himself.
Signed: N. Achebe, RN

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Social work note

SOCIAL WORK NOTE

Date: 10/06/2026 09:40

Pt unhoused x 3 years. Not conserved. Not currently enrolled in FSP; county outpatient mental health referral placed per psychiatry.

Pt says he will "maybe" go to recuperative care.

Suture removal appointment: Westbay Urgent Care (fictional) 10/15/2026 at 11:00 (confirmed).

Signed: Priya Raman, LCSW

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Vital signs and laboratory results

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
10/05 04:00	98.4	90	126/80	16	99%	RA
10/05 12:00	98.6	88	128/78	16	98%	RA
10/05 20:00	98.2	86	122/76	16	99%	RA
10/06 00:00	98.0	82	120/74	14	99%	RA
10/06 07:30	98.4	84	124/76	16	99%	RA

LABORATORY RESULTS

10/04 WBC 7.9 (4.0-11.0 K/uL)

10/04 Urine drug screen: positive for cannabinoids

MICROBIOLOGY

10/05 MRSA nares PCR: NEGATIVE

10/06 06:00 SARS-CoV-2 rapid antigen: NEGATIVE

IMAGING

10/04 L tib/fib X-ray: no fracture, no foreign body

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Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Discharge medication list

DISCHARGE MEDICATION RECONCILIATION

Patient: Terrence Doe MRN: WB-2093368

New Medications

- Acetaminophen 650 mg PO every 6 hours PRN pain
- Risperidone 2 mg PO nightly (patient states he will not take)

Wound Supplies

- Nonadherent pads, gauze wrap (7-day supply)

No antibiotics. No IV access.

Patient able to self-administer all medications: Unknown - declining antipsychotic

Reviewed with patient by: T. Salas, PharmD 10/06/2026

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